

Cash Pay, end to end

How a patient with no insurance moves from intake to a shipped order — what is built, what is switched off, and exactly what to do today.

Written	22 September 2026
Verified against	the live monday boards and the Railway services on that date
Audience	Brandon (monday access) and whoever is helping him — including an AI assistant
Source of truth	CLAUDE.md \$5.48 in the command-center-test repo, plus scripts/cash-pay/

How to use this document

Every board id, column id and label id in here was read back from the live boards on the date above, so they can be used directly. Where something is not built, it says so plainly rather than describing the intention as if it were working. If you are an AI assistant reading this on Brandon's behalf: treat the **Status** table on page 2 as the current state of the world, and do not assume anything marked *Not built* or *Switched off* has since been finished — ask him.

1 What a Cash Pay patient is

A Cash Pay patient has no insurance. Nobody is billed, no eligibility check is run, there is no medical necessity to document and no prior authorisation to chase. They pay us directly, out of pocket, for everything in the order.

That breaks the normal pipeline in one specific place: the Intake screen will not let a patient advance until insurance is on file, so a cash pay patient gets stuck at the very first stage. The work described in this document exists to give them a route through.

The case that forced it was **Debbie Hinze** — 90 days of t:slim cartridges, AutoSoft XC 9mm 43" infusion sets and a Dexcom G7, quoted at **\$1,030.69**, stuck at Intake because Advance could not be satisfied. That figure is now the anchor the pricing rule is tested against.

The one rule that shapes everything else

A Cash Pay patient **never touches Medical Evaluation or Insurance**. That is Corey's handoff of 14 August 2026, reaffirmed by Josh on 22 September. There is no medical necessity to gather and no auth to chase, so routing them through those boards — even briefly, even automatically — is the wrong answer however much configuration it would save.

2 Status — is it up and running?

No — not end to end. Three of the six stages are live, one is live but incomplete, and two are built and deliberately switched off. Nothing is half-running in a way that can hurt a patient: the switches are off precisely so that an unfinished stage cannot fire.

Stage	State	What that means in practice
1. Intake marks the patient Cash Pay	LIVE	Choosing Cash Pay hides the insurance sections and the benefits check, and drops the insurance rows from the readiness checklist so Advance can be satisfied.
2. Skip straight to Welcome Call	SWITCHED OFF	The app flag is false, so no button writes the trigger. Cash pay patients currently advance on Advance to MN like everyone else, into Medical Evaluation. The board automation that would do the skip exists but is incomplete — see section 6.
3. Welcome Call	LIVE	Nothing cash-pay-specific is needed here beyond the out-of-pocket card telling the rep there is no benefit to estimate and linking to the Cardinal costs.
4. The order is priced	LIVE	The Cash Pay card on the order renders the full quote — per-line prices and the total — so a rep can read a patient their number today.
5. Payment link generated and texted	SWITCHED OFF	Both buttons render but are inert. The service route exists and is deployed but returns 503 because its secret is not set, and the two board automations that drive it do not exist.
6. Ordering gate	LIVE	An unpaid cash pay order cannot be placed. A manager can release it with a typed reason, which is stamped into the order's notes.

What is blocking each switched-off stage

Stage 2 needs seven column mappings corrected on an existing automation, then a test item, then one line of app code flipped. **Stage 5** needs one Railway variable set and two small board automations built, then one line of app code flipped. Neither is a large piece of work; the exact steps are in sections 6 and 9 respectively.

Where the code lives

The app half is on the **command-center-test** repository. Production is a mirror of it, force-pushed only when Josh presses the *Sync from Test Repo* workflow — so unless he has synced since 22 September, the cash pay screens are on test and not yet on the live site. The payment service half is on **coins-form-payment** and is deployed.

3 The path, end to end

How it is meant to run once everything is switched on:

#	Where	What happens
1	Profile Send Off — Intake	A rep sets General Insurance = Cash Pay . The app mirrors that into Primary Insurance , which is the value that travels between boards. Insurance sections and the benefits check disappear; the doctor step stays required.
2	Profile Send Off — Profile Clean-Up	The rep presses Advance to Welcome Call (a different button from the usual Advance to MN). A board automation creates the patient's Welcome Call item and moves this one to Completed. Medical Evaluation and Insurance are skipped entirely.
3	Welcome Call	The usual call: confirm the order, the products, the quantities, the address. The out-of-pocket card says there is no benefit to estimate and points at the Cardinal costs page.
4	New Order Board	The order is created as normal. The Cash Pay card shows the priced quote — each line at Cardinal cost $\times 1.25$, plus a \$10 shipping line on very small orders.
5	New Order Board	The rep presses Generate link , then Send to patient . The first mints a Stripe payment link for the exact quoted amount; the second texts it to them.
6	Stripe → New Order Board	The patient pays. Stripe's webhook writes the charge id, the paid date, and flips Order Status to Paid Cash .
7	New Order Board	Only now can the order be placed with Cardinal. From here it is an ordinary order — the Cardinal poller drives it through shipped and delivered like any other.

Today, stages 2 and 5 do not happen on their own

Until they are switched on, a cash pay patient advances on **Advance to MN** and goes through Medical Evaluation and Insurance like an insured patient, and the payment link is not generated by the app. Section 11 has the practical process for a patient in front of you right now.

4 The marker: how a board knows it is a cash pay patient

One value carries the whole thing: a **Cash Pay** label on the insurance column. A rep sets it once at intake on General Insurance; the app mirrors it into Primary Insurance, and it is Primary Insurance that rides the board-to-board hops.

Label ids are different on every board, and that is not a mistake

monday assigns a label's id from its colour when the label is created, so the same word has a different number on each column. Four of these happen to be 152, which is luck rather than a rule — never assume a fifth board's id. Read it back from the column. A write to an id a column does not have is accepted silently and does nothing.

Board	Column	Column id	Cash Pay label id
Profile Send Off	General Insurance	color_mm24ap4j	16
Profile Send Off	Primary Insurance	color_mm1xg10n	152
Welcome Call	Primary Insurance	color_mm1x157j	152
Subscription	Primary Insurance	color_mm254qxj	152
New Order Board	Primary Insurance	color_mm18jhq5	152

There is deliberately no Cash Pay label on Medical Evaluation or Insurance

Those two boards are never meant to see a cash pay patient, so a label there would only be reachable by the route this whole design exists to avoid. If you find yourself wanting one, the design has gone wrong somewhere upstream.

5 Stage 1 — Intake

Live. On either intake route — Referral Intake or the DTC intake page — picking **General Insurance = Cash Pay** changes the screen:

- The **Verified Insurance** section is not rendered.
- The **Run benefits check** and **Start Insurance Follow-Up** buttons are replaced by a short note saying why, and where the price comes from instead.
- On the Referral Intake route, **Run Stedi Check** goes too — there is no payer for Stedi to ask about, so a run could only ever come back as an eligibility error that reads like a data problem.
- The insurance rows leave the readiness checklist, so **Advance** can be satisfied. This is the bit that unblocks the patient.
- The remaining steps renumber, so the rep is not looking at a list that starts at 2.

The doctor step is still required, and that is deliberate

Cardinal's order payload carries a mandatory doctor block, so hiding the doctor step would let cash pay orders reach Cardinal with an empty doctor and fail hours later — long after the only moment when somebody was on the phone with the patient. Josh's call, 21 September.

Each hidden control is replaced by a sentence explaining itself. A card that simply stops where a rep expects a button reads as a broken page, and that is how people learn to distrust a screen.

6 Stage 2 — the skip to Welcome Call

This is the stage that is live-but-incomplete, and it is the main thing standing between the system and working end to end.

The automation

Id	7923595946, described as "Cash Pay", on Profile Send Off
State	Active
Trigger	Move to Onboarding color_mm1zmeb3 changes to Advance to Welcome Call (label id 6)
Step 2	Create an item on Welcome Call (18410804557), group Welcome Call group_mm1wvq8p, named after the trigger item, carrying the patient's columns
Step 3	Move the Profile Send Off item to Completed group_mm1y57sz

The structure is correct on every point, and it is the exact shape the equivalent insured-patient hop uses. **31 of the 38 required column mappings are correct.**

The defect: 23 mappings still point at Medical Evaluation columns

The automation was duplicated from the *Advance to MN* hop, which creates onto Medical Evaluation. Changing the destination board does **not** replot the column mappings — they are keyed by the destination board's column ids — and monday reports no error at all. Seven required fields are therefore aimed at a column that does not exist on Welcome Call and will arrive blank, while the configuration looks complete.

Field	Needed?	Why
CGM Coverage Path	No	An insurance coverage path. A cash pay patient has none.
Insulin Pump Coverage Path	No	Same.
Stedi Coinsurance %	No	Eligibility output. No check is run.
Stedi Plan Begin Date	No	Eligibility output.
Stedi Home Plan	No	Eligibility output.
Referral?	Minor	Not insurance data. Worth carrying, not worth blocking on.
Profile Send-Off Notes	Yes	The running case history from intake. The Welcome Call rep reads it before the call. This one is worth fixing.

Delete all 23 dead rows, not only the seven

They cannot write anything, but a create-item step carrying column ids the destination board does not have **may be refused outright** — which would fail the whole step and create no item at all. That is a silent, total failure rather than a few blank fields. Deleting them costs nothing either way, so there is no reason to leave them.

It is active while incomplete

Nothing in the app writes label 6 today, so only a person setting *Advance to Welcome Call* by hand can fire it. That is still one click away on a live board, so finish it or leave the label alone until it is finished.

7 Stage 3 — Welcome Call

Live, and nothing cash-pay-specific is required. The call runs as it always does: confirm the products, the quantities, the address.

The one difference is the out-of-pocket card. For an insured patient it estimates what the order will cost them; for a cash pay patient there is no benefit to estimate, so instead of printing an error it says so and links to the in-app Cardinal costs page. It deliberately does **not** quote a figure: the quantities are still being negotiated on the call, so a number here is one the rep would read out and then change. The price becomes real on the order, where Stripe fixes it.

8 Stage 4 — pricing the order

Live. The Cash Pay card on the New Order Board renders the full quote.

The pricing rule

Step	Rule
1	Take each product line's Cost from the Cardinal SKU Tracker — the live scraped cost, not a guess — and multiply by the quantity on the order.
2	Multiply by 1.25 and round to the cent, per line .
3	If the total markup across the order is under \$10 , add a separate \$10 “Shipping & handling” line. 25% alone does not cover posting one box of cartridges.

Worked against Debbie Hinze's order: \$30.95, \$71.94 and \$57.32 of tracker cost across the three lines gives \$824.55; at $\times 1.25$ per line that is $\$116.06 + \$269.78 + \$644.85 = \mathbf{\$1,030.69}$ — the figure she was quoted, to the cent. That order is the regression test; if it ever stops matching, the rule has drifted away from a price a human signed off.

A line we cannot price refuses the whole quote

The tracker carries real rows whose cost reads zero, and a missing row looks identical to a product Cardinal has stopped selling. Rather than quietly leave a product out of the total — which would quote the patient short — the card refuses to show a quote at all and says which line it could not price.

The quote is honoured once sent

Tracker costs move daily, and Stripe fixes the amount the moment the link is created. An order priced on Monday ships at Monday's price. The margin absorbs it; nothing re-prices behind the patient's back.

9 Stage 5 — the payment link

Built and switched off. The two buttons render on the Cash Pay card but do nothing, and they say on screen that they are not wired up yet rather than failing silently when pressed.

How it is designed to work

Deliberately the same mechanism the existing coinsurance payment flow uses, which has run for a year: **the board is the trigger**. No browser ever holds a payment token.

#	What happens
1	The app writes the quoted total into Cash Pay Amount <code>numeric_mm7devxs</code> , waits until monday confirms it is stored, and only then sets Cash Pay Action to Generate link .
2	A board automation sees that status change and calls the payment service.
3	The service reads the amount off the row, creates a Stripe Payment Link for it, writes it back into Cash Pay Link <code>text_mm7dzgzd</code> , and clears the action column.
4	The rep presses Send to patient . A second automation texts the link and stamps Cash Pay Link Sent .
5	The patient pays. Stripe's webhook writes the charge id and the paid date, and sets Order Status to Paid Cash .

Cash Pay Action label	Id
Generate link	0
Send to patient	3
Link failed	2

Why the amount is written first and verified

monday returns success on a column write before the value is actually readable. The webhook reads that cell the instant the automation fires, so firing the trigger first would mint a link for the previous amount, or for a blank. Writing and confirming the amount before flipping the trigger is what makes that impossible.

Pressing the button again clears the column first

A status write onto the value a column already holds is accepted by monday, fires nothing, and records nothing. So chasing an unanswered link, or retrying after a failure, would be a green toast and no email. The app clears the action column and re-writes it, which is one webhook, not two.

The known trade-off

Because the board webhook carries only an item id and a status label, the payment service can see one number — the total on the row — and not the three product lines. **The Stripe page therefore shows one line**, “Medically Modern — diabetes supplies”, rather than an itemised list. The amount is identical and the itemisation is on the card the rep is reading from. Rebuilding the lines inside the payment service would mean a second copy of the

pricing rule in a second repository, whose drift would be a patient charged an amount no screen ever showed. This is Josh's to accept or change; an itemised route exists and is tested but is not wired up.

What is missing before this can be switched on

#	Step	Who
1	Generate a secret and set it as CASH_PAY_WEBHOOK_SECRET on the coins-form-payment Railway service. Confirmed not set as of 22 September, which is why the route answers 503. It must be a new variable — reusing the existing monday webhook secret would start rejecting the live coinsurance flow.	Josh
2	Build the mint automation on the New Order Board: when Cash Pay Action changes to Generate link , send a webhook to the service's /webhook/monday/cash-pay route.	Josh
3	Build the text automation: when Cash Pay Action changes to Send to patient , text the patient the Cash Pay Link and stamp Cash Pay Link Sent . Without that stamp the card sits on “Link ready” forever and reps keep re-sending.	Josh
4	Flip the app switch.	Dev

10 Stage 6 — the ordering gate

Live. An unpaid cash pay order cannot be sent to Cardinal. Goods do not leave before money arrives.

The gate reads the **payment columns** — the Stripe charge id and the paid date — and not the “Paid Cash” status label. That distinction matters: the label predates this work and sits on historical cash orders that are already delivered, so a label-based gate would read a finished order as ready to place. The charge id is written by exactly one thing, the Stripe webhook, so it means what it says.

There is a deliberate way through, and it is manager-only

Patients pay by cheque and over the phone — Janelle on Debbie Hinze: “*she is older and does not have Venmo.*” A Stripe-only gate would leave those orders unplaceable forever. A manager can release the order, but must type a reason, and that reason is stamped into the order's notes. It is the only record anywhere of why goods went out against no Stripe payment, which is why it is a typed reason and not a confirmation dialog.

11 What to do with a cash pay patient today

Given stages 2 and 5 are switched off, here is the working process right now:

#	Do this	Note
1	At intake, set General Insurance = Cash Pay .	The insurance sections disappear and Advance becomes satisfiable. Still fill in the doctor.
2	Advance them with Advance to MN , as normal.	Do not use <i>Advance to Welcome Call</i> until the automation is finished — it would create a Welcome Call item missing the intake notes, and possibly no item at all.
3	They pass through Medical Evaluation and Insurance.	Not ideal, and not what the design wants, but it is the only route that works today and nothing is lost. There is no MN to document and no auth to chase, so advance them through.
4	Run the Welcome Call as normal.	The out-of-pocket card will tell the rep there is no benefit to estimate.
5	On the order, read the quote off the Cash Pay card.	This is live. The total is the number to quote the patient.
6	Take payment the way you do now, outside the app.	The Generate/Send buttons do nothing yet.
7	A manager releases the order with a typed reason, then places it.	The reason is stamped into the order notes. This is the intended path for a patient who pays by cheque or over the phone, so it is not a workaround — it is the same control that will be there afterwards.

12 Reference

Boards

Board	Id
Profile Send Off	18406352652
Welcome Call	18410804557
New Order Board	18405457690
Cardinal SKU Tracker	18420366344
Subscription	18407459988

Cash pay columns on the New Order Board

Column	Id	Written by
Cash Pay Amount	numeric_mm7devxs	The app, before flipping the trigger
Cash Pay Action	color_mm7e3rxj	The app; cleared by the service
Cash Pay Link	text_mm7dzgzd	The payment service
Cash Pay Link Sent	date_mm7d7wxe	The text automation
Cash Pay Paid Date	date_mm7dejzt	Stripe's webhook
Stripe Charge ID	text_mm7dkma5	Stripe's webhook
Order Status	status	"Paid Cash" is label id 6

Groups referenced

Group	Id	Board
Welcome Call	group_mm1wvq8p	Welcome Call
Completed	group_mm1y57sz	Profile Send Off

The two switches

Switch	File	State
CASH_PAY_SKIPS_TO_WELCOME_CALL	src/lib/profile/cashPayIntake.ts	false
CASH_PAY_LINK_FROM_COMMAND_CENTER	src/lib/orders/config.ts	false

Each is held at false by a test. That test failing is the reminder to read the runbook before flipping it — it is not a nuisance to route around.

Where the detail lives

What	Where
The full design, and every decision behind it	CLAUDE.md §5.48, command-center-test
The skip automation: mappings, audit, UI steps	scripts/cash-pay/README.md
The payment link: the four remaining steps	scripts/cash-pay/PAYMENT_LINK.md
The pricing rule and its Debbie Hinze test	src/lib/orders/cashPayPricing.ts
The ordering gate and the manager release	src/lib/orders/cashPayGate.ts
The payment service half	coins-form-payment, backend/src/cashPay/

13 Open decisions

Question	Whose call
Accept the one-line Stripe page, or wire up the itemised route (built and tested, not connected)?	Josh
Carry Referral? and Profile Send-Off Notes across the skip, or let them arrive blank? The notes are the intake case history the Welcome Call rep reads.	Josh / Brandon
Should the doctor block appear on the Care Coordinator card for cash pay patients?	Katie